Critical Care Clinical Psychology Service - YG

PATIENT Referral Form

<table>
<tr><td colspan="2">Client name:

Date of Birth:</td><td>Hospital No:

NHS No:</td></tr>
<tr><td>Address :</td><td>Patient's status
☐ ITU ☐ HDU
☐ Ward…. ☐ Outpt.</td><td>Telephone Numbers :
Home :
Mobile :</td></tr>
<tr><td>When was the ICU admission?</td><td colspan="2">What was the ICU admission for?</td></tr>
<tr><td colspan="3">ICU Clinical Psychology Intervention Please outline main concerns and hopes regarding assessment / intervention:

IPAT Score (for help with IPAT please see overleaf) : ☐</td></tr>
<tr><td colspan="3">Any other significant information? (e.g. family circumstances; previous Health / MH needs, concern regarding current communication / cognition / orientation / engagement with care, CC distress)

Other Measures completed? PHQ-9 ☐ GAD-7 ☐ TSQ ☐</td></tr>
<tr><td>Is this a self -referral?

Yes / No

If NO, has the person named consented to this referral?

YES / NO</td><td>If referred by a professional – name and contact details of referring person :

Date of referral:</td><td>Referral Received:

Date :

Accepted : YES / NO

Actions :</td></tr>
</table>

Please return forms to: **Dr Bethan Henderson, Clinical Psychologist**,

c/o please scan the form and email it to Bethan.Henderson2@wales.nhs.uk

When might a referral to the Critical Care Clinical Psychologist be appropriate?

When on the Critical Care ward and there is evidence of ICU distress as indicated by :

- ☐ A score of 9 or more on the Intensive Care Psychological Distress Tool (IPAT) (NB scores of 7-9 would indicate Enhanced Psychological Care required by ward based staff).
- ☐ Patient / Family request Clinical Psychology Review
- ☐ Consultation with bedside staff about the psychological needs and care of this patient

IPAT	0	1	2
Has it been hard to communicate?	No	Yes, a bit	Yes, a lot
Has it been difficult to sleep?	No	Yes, a bit	Yes, a lot
Have you been feeling tense?	No	Yes, a bit	Yes, a lot
Have you been feeling sad?	No	Yes, a bit	Yes, a lot
Have you been feeling panicky?	No	Yes, a bit	Yes, a lot
Have you been feeling hopeless?	No	Yes, a bit	Yes, a lot
Have you felt disorientated (not sure where you are)?	No	Yes, a bit	Yes, a lot
Have you had hallucinations (seen or heard things that you suspect not really there)?	No	Yes, a bit	Yes, a lot
Have you felt that people were deliberately trying to hurt you?	No	Yes, a bit	Yes, a lot
Do upsetting memories of intensive care keep coming to mind	No	Yes, a bit	Yes, a lot

After discharge, when there is:

- ☐ A clinical concern regarding psychological well-being during follow up clinic – e.g. concern around depression, anxiety, PTSD and / or cognitive impairment **that appears to be directly related to their critical care admission**
- ☐ Patient / Family request within 12 months of discharge from ICU

EXCLUSIONS – patients meeting any of the criteria below will not be accepted

- Patients with a pre-existing mental health problem (e.g CSA, Psychosis, Severe and enduring depression or anxiety, Eating Disorder) that either are open to or eligible for a service from AMH / CMHT / CAMHS / CHP
- Patients with current debilitating drug and / or alcohol misuse
- Pre existing severe cognitive impairment or brain injury
- Patients exceeding 12 months since discharge from Critical Care

RISK

Please note the Clinical Psychology Provision for Critical Care is only a small resource. We are therefore unable to respond to urgent concerns around risk / patient safety. If you have a concern about keeping yourself (or a patient safe) we would advise you to contact your GP, ED or if hospital based, your Liaison Psychiatry Team or Safeguarding Team.